FOLFOXIRI Therapy

INDICATIONS FOR USE:

INDICATION	ICD10	Regimen Code	HSE approved reimbursement status*
Treatment of metastatic colorectal cancer[i]	C18-20	00555a	N/A

*This applies to post 2012 indications

TREATMENT:

The starting dose of the drugs detailed below may be adjusted downward by the prescribing clinician, using their independent medical judgement, to consider each patient's individual clinical circumstances.

Irinotecan, oxaliplatin, folinic acid are administered by IV infusion on Day 1 of each cycle.
5-fluorouracil is administered by a continuous IV infusion over 48 hours beginning on Day 1 of each cycle.

Cycle frequency and duration:

Each cycle is 14 days.
Treatment should continue until disease progression or unacceptable toxicity.

Facilities to treat anaphylaxis MUST be present when systemic anti-cancer therapy (SACT) is administered.

Admin. Order	Day	Drug	Dose	Route	Diluent & Rate	Cycle
1	1	Irinotecan	165mg/m^2	IV infusion	250mL NaCl 0.9% over 90 minutes	All cycles
2	1	Oxaliplatin[a]	85 mg/m^2	IV infusion	500mL glucose 5% over 2 hours	All cycles
2	1	Folinic Acid (Calcium leucovorin)[b]	200mg/m^2	IV infusion	250mL glucose 5% over 2 hours, administer concurrently with oxaliplatin	All cycles
3	1	5-Fluorouracil[c]	3200mg/m^2	Continuous IV infusion	Over 48 hours in NaCl 0.9% (equivalent to 1600mg/m^2/day)	All cycles

[a]Oxaliplatin is incompatible with NaCl 0.9%. Do not piggyback or flush lines with NaCl 0.9%.
For oxaliplatin doses ≤ 104mg use 250mL glucose 5%.
Increase infusion rate time to 4 – 6 hours in case of laryngopharyngeal dysaesthesia reaction.
Oxaliplatin administration must always precede the administration of 5-Fluorouracil.
Oxaliplatin may be given at the same time as folinic acid (calcium leucovorin) using a Y connector.

[b]Folinic Acid *(Calcium Leucovorin)* must be administered prior to 5-Fluorouracil. It enhances the effects of 5-Fluorouracil by increasing 5-Fluorouracil binding to the target enzyme thymidylate synthetase

[c]See dose modifications section for patients with identified partial dihydropyrimidine dehydrogenase (DPD) deficiency

Note: Administration volumes and fluids have been standardised to facilitate electronic prescribing system builds.

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ELIGIBILITY:

- Indication as above
- ECOG ≤ 2 (adults under 70 years)
- ECOG < 1 (adults 71-75)
- Adequate haematological, renal and liver status

CAUTIONS:

- Previous pelvic radiotherapy
- Recent MI, uncontrolled angina, hypertension, cardiac arrhythmias, CHF
- In patients with baseline greater than 3 loose bowel movements (BM) per day (in patients without colostomy or ileostomy)
- Symptomatic peripheral neuropathy

EXCLUSIONS:

- Hypersensitivity to irinotecan, oxaliplatin, 5-Fluorouracil or any of the excipients
- Baseline neutrophils < 2 x 10^9/L and/or platelet count < 100 x 10^9/L
- Bilirubin > 3 x ULN
- Chronic bowel disease and/or bowel obstruction
- Pregnancy
- Breastfeeding
- Severe bone marrow failure
- CNS metastases
- Known complete DPD deficiency

PRESCRIPTIVE AUTHORITY:

The treatment plan must be initiated by a Consultant Medical Oncologist.

TESTS:

Baseline tests:

- FBC, renal and liver profile
- ECG (if patient has compromised cardiac function)
- DPD testing prior to first treatment with 5-Fluorouracil using phenotype and/or genotype testing unless patient has been previously tested
    - In patients with moderate or severe renal impairment, blood uracil levels used for DPD phenotyping should be interpreted with caution, as impaired kidney function can lead to increased uracil blood levels. Consequently, there is an increased risk for incorrect diagnosis of DPD deficiency, which may result in under dosing of 5-Fluorouracil or other fluoropyrimidines, leading to reduced treatment efficacy. Genotype testing for DPD deficiency should be considered for patients with renal impairment.

Regular tests:

- FBC, renal and liver profile prior to each cycle
- Evaluate for peripheral neuropathy every cycle prior to proceeding with treatment

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Disease monitoring:
Disease monitoring should be in line with the patient's treatment plan and any other test/s as directed by the supervising Consultant.

DOSE MODIFICATIONS:

- Consider a reduced starting dose in patients with identified partial DPD deficiency
    - Initial dose reduction may impact the efficacy of treatment
    - In the absence of serious toxicity, subsequent doses may be increased with careful monitoring
- Any dose modification should be discussed with a Consultant

Haematological:

- Treatment is not administered unless ANC ≥ 1.0 x 10^9/L and platelets ≥ 75 x 10^9/L
- If levels are below this at Day 1 treatment may be delayed for 1-2 weeks
- If no recovery in 2 weeks consideration should be given to discontinuing the treatment

Table 1: Dose modification of FOLFOXIRI based on Day 1 Absolute Neutrophil Count (ANC)

ANC < 1.0 x 10^9/L	Irinotecan	Oxaliplatin	5-Fluorouracil
1st occurrence	Reduce dose to 150mg/m^2	Maintain full dose	Reduce to 75% of the original dose
2nd occurrence	Maintain 150mg/m^2 dose	Reduce to 60mg/m^2	Reduce to 50% of the original dose
3rd occurrence	DISCONTINUE TREATMENT		

Table 2: Dose modification of FOLFOXIRI based on Day 1 Platelet Count

Platelets < 75 x 10^9/L	Irinotecan	Oxaliplatin	5-Fluorouracil
1st occurrence	Maintain full dose	Reduce to 60mg/m^2	Reduce to 75% of the original dose
2nd occurrence	Reduce dose to 150mg/m^2	Maintain at 60mg/m^2	
3rd occurrence	DISCONTINUE TREATMENT		

Table 3: Dose modification of FOLFOXIRI based on low nadir blood counts or in case of infection

Platelets < 50 x 10^9/L	Irinotecan	Oxaliplatin	5-Fluorouracil
1st occurrence	Maintain full dose	Reduce to 60mg/m^2	Reduce to 75% of the original dose
2nd occurrence	Reduce dose to 150mg/m^2	Maintain at 60mg/m^2	Reduce to 50% of the original dose
3rd occurrence	DISCONTINUE TREATMENT		

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Renal and Hepatic Impairment:

Table 4: Recommended dose modifications for patients with renal and hepatic impairment

Drug	Renal Impairment		Hepatic Impairment	
	CrCl (mL/minute)	**Dose**		
Irinotecan	≥ 10	No need for dose adjustment is expected	Irinotecan is contraindicated in patients with bilirubin levels > 3 x ULN	
	< 10	Start with 50-66% of the original dose, increase if tolerated		
	Haemodialysis	Start with 50-66% of the original dose, increase if tolerated		
Oxaliplatin	**CrCl (mL/minute)**	**Dose**	No dose adjustment is needed	
	≥ 30	No dose adjustment is needed		
	< 30	Consider 50% of the original dose		
	Haemodialysis	Consider 50% of the original dose. Haemodialyse within 1.5 hours after administration		
5-Fluorouracil	No need for dose adjustment is expected		**Severity**	**Dose**
	Haemodialysis	No need for dose adjustment is expected	Mild and moderate (without renal impairment)	No need for dose adjustment is expected
			Severe	Not recommended

Irinotecan: Renal – Giraud et al 2023. Hepatic – SmPC
Oxaliplatin: Renal and hepatic – Giraud et al 2023.
5-fluorouracil: Renal and hepatic – Giraud et al 2023.

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Management of adverse events:

Table 5: Recommended dose modification schedule based on non-haematological, non-neurological toxicities

Prior to a Cycle (DAY 1)	Irinotecan	Dose Level for Subsequent Cycles	
		Oxaliplatin	5-Fluorouracil
Diarrhoea			
• **1st occurrence** ○ Grade 3 or 4 ○ With fever and/or Grade 3 or 4 neutropenia	Reduce dose to 150mg/m^2		Reduce to 75% of original dose
• **2nd occurrence**		Reduce dose to 60mg/m^2	Reduce to 50% of the original dose
• **3rd occurrence**	DISCONTINUE TREATMENT		
Mucositis or hand-foot syndrome Grade 3 or 4			Reduce to 75% of original dose
Other toxicity • ≥ Grade 2 (except alopecia and anaemia)	Consider dose reduction		
• Transient grade 3 paresthesias/dysesthesias or transient grade 2 symptoms lasting > 7 days		Decrease oxaliplatin by 25%	
• Grade 4 or persistent grade 3		Discontinue oxaliplatin	
Laryngo-pharyngeal dysaesthesia		Increase infusion time from 2 to 6 hours	

SUPPORTIVE CARE:

EMETOGENIC POTENTIAL:

- As outlined in NCCP Classification Document for Systemic AntiCancer Therapy (SACT) Induced Nausea and Vomiting Available on the NCCP website

This regimen poses an overall high risk of emesis (Refer to local policy)

Irinotecan: Moderate
Oxaliplatin: Moderate
5-Fluorouracil: Low

For information:
Within NCIS regimens, antiemetics have been standardised by the Medical Oncologists and Haemato-oncologists and information is available in the following documents:

- NCCP Supportive Care Antiemetic Medicines for **Inclusion in NCIS** (Medical Oncology) - Available on the NCCP website
- NCCP Supportive Care Antiemetic Medicines for **Inclusion in NCIS** (Haemato-oncology) - Available on the NCCP website

PREMEDICATIONS:

- Prophylactic atropine sulphate 250 micrograms subcutaneously prior to irinotecan infusion. Atropine should not be used in patients with glaucoma (See regimen specific complications below).

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OTHER SUPPORTIVE CARE:

- Management of PPE **(refer to local policy)**
- Anti-diarrhoeal treatment **(refer to local policy)**

Patients should be made aware of the risk of delayed diarrhoea occurring more than 24 hours after the administration of irinotecan and at any time before the next cycle.

- As soon as the first liquid stool occurs, the patient should start drinking large volumes of beverages containing electrolytes and an appropriate anti-diarrhoeal therapy must be initiated immediately
- The currently recommended anti-diarrhoeal treatment consists of high doses of loperamide (4 mg for the first intake and then 2mg every 2 hours).
- This therapy should continue for 12 hours after the last liquid stool and should not be modified.
- In no instance should loperamide be administered for more than 48 consecutive hours at these doses, because of the risk of paralytic ileus, nor for less than 12 hours.

Patients should be warned about the potential for dizziness or visual disturbances which may occur within 24 hours following the administration of irinotecan, and advised not to drive or operate machinery if these symptoms occur.

ADVERSE EFFECTS:

- Please refer to the relevant Summary of Product Characteristics (SmPC) for full details of adverse effects.
- Please note the NCCP National SACT Regimens do not provide information regarding black triangle (▼) status. Please refer to the relevant Summary of Product Characteristics (SmPC) for this information.

REGIMEN SPECIFIC COMPLICATIONS:

Oxaliplatin

- **Platinum Hypersensitivity**: Special surveillance should be ensured for patients with a history of allergic manifestations to other products containing platinum. In case of anaphylactic manifestations the infusion should be interrupted immediately and an appropriate symptomatic treatment started. Re-administration of oxaliplatin to such patients is contraindicated.
- **Laryngopharyngeal dysaesthesia:** An acute syndrome of laryngopharyngeal dysaesthesia occurs in 1% - 2% of patients and is characterised by subjective sensations of dysphagia or dyspnoea/feeling of suffocation, without any objective evidence of respiratory distress (no cyanosis or hypoxia) or of laryngospasm or bronchospasm. Symptoms are often precipitated by exposure to cold. Although antihistamines and bronchodilators have been administered in such cases, the symptoms are rapidly reversible even in the absence of treatment. Prolongation of the infusion helps to reduce the incidence of this syndrome.
- **Gastrointestinal toxicity:** It manifests as nausea and vomiting and warrants prophylactic and/or therapeutic anti-emetic therapy. Dehydration, paralytic ileus, intestinal obstruction, hypokalemia, metabolic acidosis and renal impairment may be caused by severe diarrhoea/emesis particularly when combining oxaliplatin with 5-Fluorouracil.
- **Venous occlusive disease:** A rare but serious complication that has been reported in patients (0.02%) receiving oxaliplatin in combination with 5-Fluorouracil. This condition can lead to hepatomegaly, splenomegaly, portal hypertension and/or esophageal varices. Patients should be instructed to report any jaundice, ascites or hematemesis immediately.
- **Haemolytic Ureamic Syndrome (HUS):** Oxaliplatin therapy should be interrupted if HUS is

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suspected: hematocrit is less than 25%, platelets less than 100,000 and creatinine greater than or equal to 135 micromol/L. If HUS is confirmed, oxaliplatin should be permanently discontinued.

Irinotecan

- **Acute cholinergic syndrome:** If acute cholinergic syndrome appears (defined as early diarrhoea and various other symptoms such as sweating, abdominal cramping, lacrimation, myosis and salivation) atropine sulphate (250 micrograms subcutaneously) should be administered unless clinically contraindicated. Caution should be exercised in patients with asthma. In patients who experienced an acute and severe cholinergic syndrome, the use of prophylactic atropine sulphate is recommended with subsequent doses of irinotecan**.**
- **Diarrhoea -** Irinotecan induced diarrhoea can be life threatening and requires immediate management.
    - Diarrhoea (early onset) - see acute cholinergic syndrome above.
    - Diarrhoea (late onset):
        - Irinotecan induced diarrhoea can be life threatening and requires immediate management.
        - In monotherapy, the median time of onset of the first liquid stool was on day 5 after the infusion of irinotecan.
        - Patients with an increased risk of diarrhoea are those who had previous abdominal/pelvic radiotherapy, those with baseline hyperleucocytosis, those with performance status ≥ 2 and women.
        - In patients who experience severe diarrhoea, a reduction in dose is recommended for subsequent cycles.
        - The SmPC (10) provides guidelines on when hospitalisation for the management of diarrhoea is recommended.
- **Gilbert's Syndrome:** Increases the risk of irinotecan-induced toxicity. A reduced initial dose should be considered for these patients.
- **Respiratory disorders:** Severe pulmonary toxicity has been reported rarely. Patients with risk factors should be monitored for respiratory symptoms before and during irinotecan therapy.

5-Fluorouracil

- **Myocardial ischaemia and angina:** Cardiotoxicity is a serious complication during treatment with 5-Fluorouracil. Patients, especially those with a prior history of cardiac disease or other risk factors, treated with 5-Fluorouracil, should be carefully monitored during therapy.
- **DPD deficiency:** DPD is an enzyme encoded by the DPYD gene which is responsible for the breakdown of fluoropyrimidines. Patients with DPD deficiency are therefore at increased risk of fluoropyrimidine-related toxicity, including for example stomatitis, diarrhoea, mucosal inflammation, neutropenia and neurotoxicity. Treatment with 5-Fluorouracil, capecitabine or tegafur-containing medicinal products is contraindicated in patients with known complete DPD deficiency. Consider a reduced starting dose in patients with identified partial DPD deficiency. Initial dose reduction may impact the efficacy of treatment. In the absence of serious toxicity, subsequent doses may be increased with careful monitoring. Therapeutic drug monitoring (TDM) of 5-Fluorouracil may improve clinical outcomes in patients receiving continuous 5-Fluorouracil infusions.
- **Hand-foot syndrome (HFS),** also known as palmar-plantar erythrodysaesthesia (PPE), has been reported as an unusual complication of high dose bolus or protracted continuous therapy for 5-Fluorouracil (see table 5 for dose modifications).

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DRUG INTERACTIONS:

- Current drug interaction databases and SmPCs should be consulted for more information.

REFERENCES:

1. Falcone A et al. Phase III Trial of Infusional Fluorouracil, Leucovorin, Oxaliplatin, and Irinotecan (FOLFOXIRI) Compared With Infusional Fluorouracil, Leucovorin, and Irinotecan (FOLFIRI) As First-Line Treatment for Metastatic Colorectal Cancer: The Gruppo Oncologico Nord Ovest (GONO)2007 J Clin Oncol *25:1670-1676*
2. Souglakos J et al. FOLFOXIRI (folinic acid, 5-fluorouracil, oxaliplatin and irinotecan) vs FOLFIRI (folinic acid, 5-fluorouracil and irinotecan) as first-line treatment in metastatic colorectal cancer (MCC): a multicentre randomised phase III trial from the Hellenic Oncology Research Group (HORG) British Journal of Cancer (2006) 94, 798 – 805
3. Montagnan et al. A systematic review of FOLFOXIRI chemotherapy for the first line treatment of metastatic colorectal cancer: improved efficacy at the cost of increased toxicity. 2011 The Association of Coloproctology of Great Britain and Ireland. 13, 846–854
4. Conroy T, Desseigne F et al. FOLFOXIRI versus Gemcitabine for Metastatic Pancreatic Cancer. NEJM 2011;364:1817-1825.
5. Giraud E L, Lijster B D, et al. Dose recommendations for anticancer drugs in patients with renal or hepatic impairment: an update. Available at: https://pubmed.ncbi.nlm.nih.gov/37269847/
6. NCCP Classification Document for Systemic Anti-Cancer Therapy (SACT) Induced Nausea and Vomiting. V6 2025. Available at: https://assets.hse.ie/media/documents/Classification_Document_for_Systemic_Anti-_Cancer_Therapy_SACT_Induced_Nausea__pWgDCrm.pdf
7. EMA Direct Healthcare Professional Communication. 5-Fluorouracil (i.v.), capecitabine and tegafur containing products: Pre-treatment testing to identify DPD-deficient patients at increased risk of severe toxicity. Accessed 04/05/2026. Available at: ema.europa.eu/en/documents/dhpc/direct-healthcare-professional-communication-dhpc-5-fluorouracil-iv-capecitabine-and-tegafur-containing-products-pre-treatment-testing-identify-dpd-deficient-patients-increased-risk-severe-toxicity_en.pdf
8. Oxaliplatin (Accord) 5mg/mL Concentrate for Solution for Infusion. Summary of Product Characteristics. Last updated: 14/03/2026. Accessed 05/05/2026. Available at: https://assets.hpra.ie/products/Human/27562/Licence_PA2315-114-001_07102025144015.pdf
9. 5-Fluorouracil (Accord) 50mg/mL Solution for Injection or Infusion. Summary of Product Characteristics. Last updated 04/02/2026. Accessed 05/05/2026. Available at: https://assets.hpra.ie/products/Human/27540/Licence_PA2315-091-001_05032025160439.pdf
10. Irinotecan (Fresenius Kabi) 20mg/mL Concentrate for Solution for Infusion. Summary of Product Characteristics. Last updated 24/04/2026. Accessed 05/05/2026. Available at: https://assets.hpra.ie/products/Human/28074/Licence_PA2059-044-001_02102025132406.pdf

From 19/08/2026, this regimen will be reviewed on an 'As Required' basis. Please refer to the version control box for information, and the footer for date of last review. If you wish to suggest any updates to this regimen, please contact oncologydrugs@cancercontrol.ie

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Version	Date	Amendment	Approved By
1	18/03/2019		Prof Maccon Keane
2	12/02/2020	Standardisation of treatment table. Updated exclusions and drug interactions. Updated recommended dose modifications for oxaliplatin in renal impairment.	Prof Maccon Keane
3	3/9/2020	Updated exclusion criteria, baseline testing, dose modifications and adverse events with respect to DPD deficiency as per DHPC from HPRA June 2020 Updated Adverse events regarding palmar-plantar erythrodysaesthesia	Prof Maccon Keane
4	10/03/2021	Reviewed. Amended exclusions and emetogenic potential.	Prof Maccon Keane
5	21/07/2021	Clarification of administration details	Prof Maccon Keane
6	05/09/2022	Updated emetogenic potential.	Prof Maccon Keane
6a	23/11/2023	Formatting changes and grammatical corrections.	NCCP
6b	13/03/2025	Additional wording added to baseline testing section.	NCCP
7	19/08/2026	Regimen reviewed. Updated: • Regimen review schedule to an 'as required' basis • ICD-10 codes to 18-20 • NCCP Standardisation • Exclusions • Haematological dose modifications (ANC cut offs updated to 1.0 $x10^9$/L) • Renal and hepatic impairment updated to Giraud et al 2023 • Supportive care • Adverse effects • Regimen specific complications • Drug interactions • References • Unlicensed footnote added	Prof Maccon Keane

Comments and feedback welcome at oncologydrugs@cancercontrol.ie.

[i] This is an unlicensed indication for the use of irinotecan in Ireland. Patients should be informed of this and consented to treatment in line with the hospital's policy on the use of unlicensed medication and unlicensed or "off label" indications. Prescribers should be fully aware of their responsibility in communicating any relevant information to the patient and also ensuring that the unlicensed or "off label" indication has been acknowledged by the hospital's Drugs and Therapeutics Committee, or equivalent, in line with hospital policy

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